In Vantaa-Kerava, 85% of non-urgent GP visits come more than a week after people ask. In Päijät-Häme it's a third.

85.4% Vantaa-Kerava

32.7% Päijät-Häme

Finland 54.7%

THL Sotkanet 6411, 2025 (share of non-urgent GP visits kept more than 7 days after the need was assessed)

Where the GP is the first stop, 75 to 85 percent of cancer patients start there. Yet no one shows a county where its own first step breaks

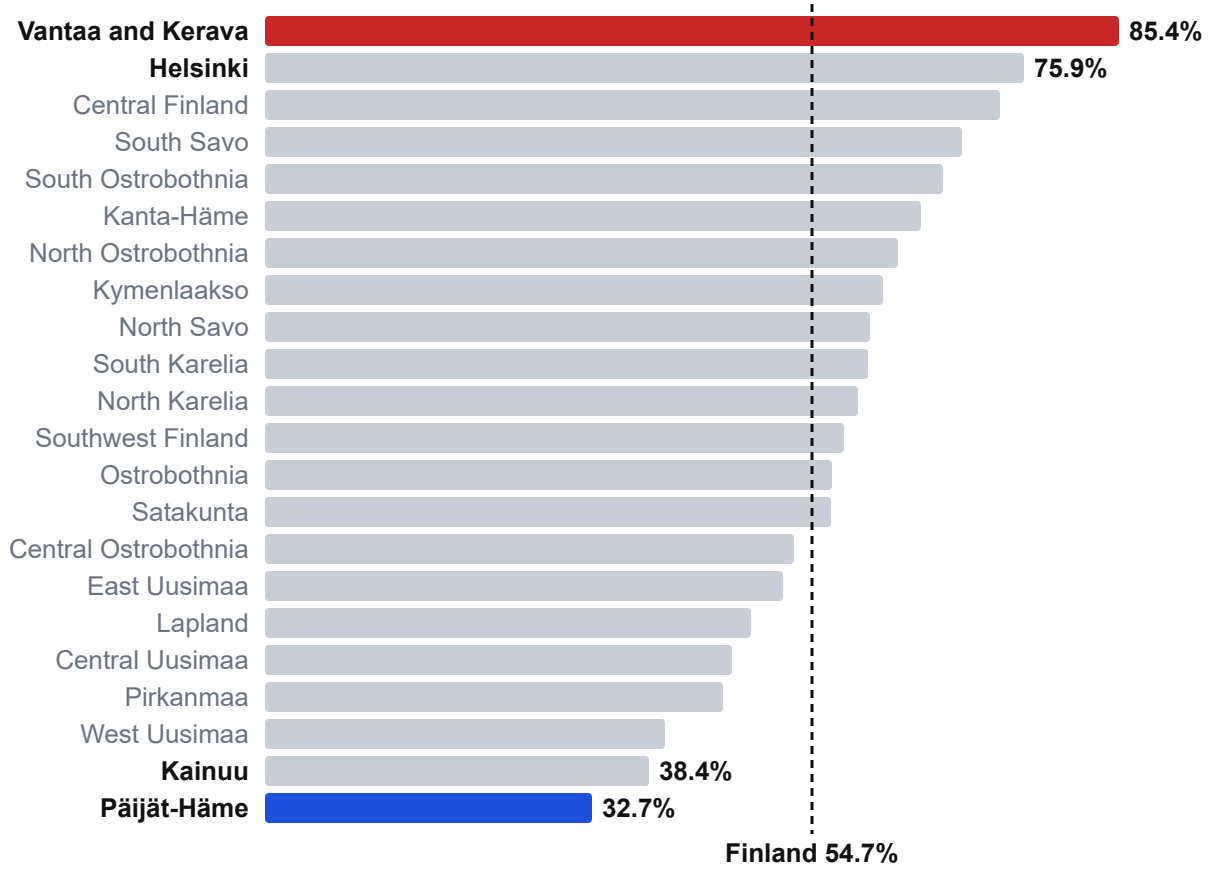


Context: National cancer strategy 2026-2035, equal care everywhere.

Steps 1 and 2 are the two steps FirstLine measures. GP as first stop: Vedsted & Olesen, Br J Cancer 2015 (PMC4385978); Danish and UK data, not Finnish. Strategy: Finnish national cancer strategy 2026-2035, implementation roadmap 2026-2031 (STM)

Counties break at different steps, so one national fix won't work

Non-urgent GP visits kept more than 7 days after the need was assessed, % of visits, 2025



Kainuu

Quick to see a GP (38.4%, Finland 54.7%), slower from GP to specialist: 75.2% of referrals processed within 3 days (Finland 76.8%).

Vantaa-Kerava

Slow to see a GP (85.4%). No public referral data.

THL Sotkanet 6411, 2025 (bars). Sotkanet 13, 2025 (referrals processed within 3 days): lowest South Ostrobothnia 68.9%, highest Kanta-Häme 88.7% among areas with data.

For each county, FirstLine shows where the delay is, and says openly where public data is missing



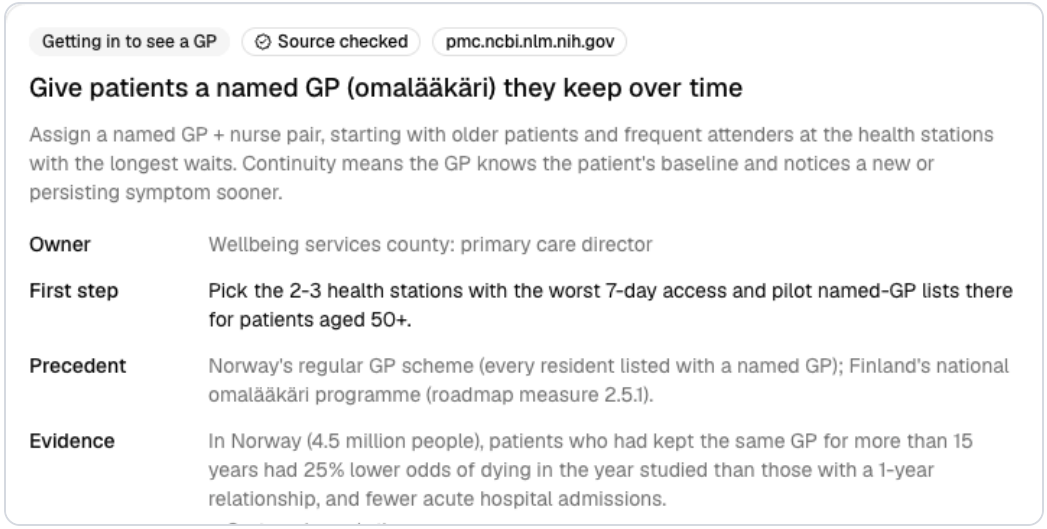
screenshot: main 84d76f2, local build; retake from live URL

- Worse than Finland
- Finland's level or better, still a wait
- No public data

Vantaa and Kerava county page. THL Sotkanet 6411, 2025; Sotkanet 13, 2025 (no value published for this county)

Every county gets 2-3 fixes that already worked elsewhere, each with an owner and a first step for Monday

Vantaa-Kerava: the delay is getting in to a GP



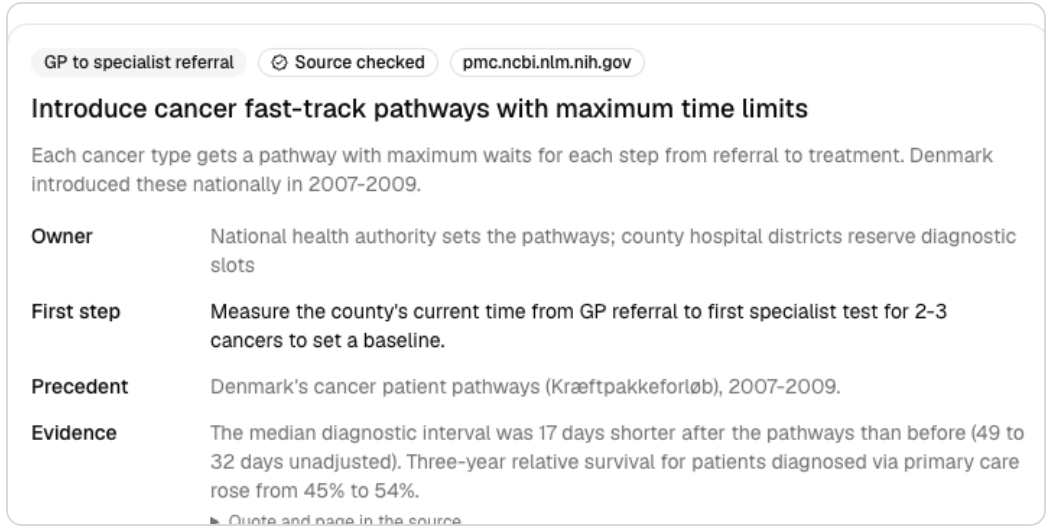
screenshot: main 84d76f2, local build; retake from live URL

25%

lower odds of dying in the year studied for patients who kept the same GP for 15+ years (Norway), and fewer hospital admissions.

Sandvik et al., Br J Gen Pract 2022 · ev-gp-continuity

South Ostrobothnia: the delay is after the GP



screenshot: main 84d76f2, local build; retake from live URL

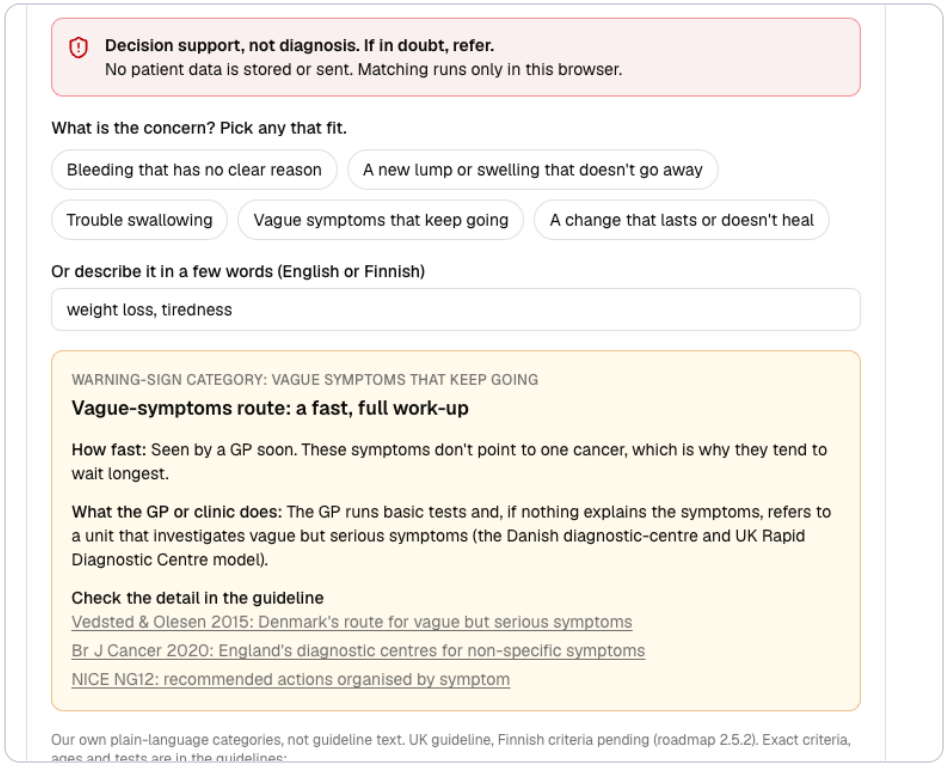
17 days

shorter median diagnostic interval (adjusted) after Denmark's fast-track cancer pathways.

Jensen et al., BMC Cancer 2015 · ev-ref-denmark-cpp

Each card shows owner, first step, precedent and a source. 2 of these fixes are already in Finland's 2026-2031 cancer roadmap (measures 2.5.1, 2.5.2).

For the GP: symptoms in, guideline warning signs and the fastest referral route out



screenshot: main 84d76f2, local build; retake from live URL

Decision support, not diagnosis

> Type "weight loss, tiredness" (or pick a category). The helper shows the vague-symptoms route and links the guidelines. It does not copy their text.

> Matching runs in the browser. No patient data is stored or sent.

Links to the guidelines (NICE NG12, UK; Käypä hoito); Finnish national criteria pending (roadmap 2.5.2)

Checked data shows where care breaks; Exa shows what's happening about it this week, with a link for every claim

County news strip screenshot

newest card with date and site; never edit a date

found live - unverified | checked evidence | TODO | dated screenshot (owner: Phương, N4; redo Sun 07:00)

Judges can ask their own question; answers come only from linked results, or "nothing found".

A county director can see their bottleneck and start the first step in two minutes, using public data only

- Every number is sourced. No patient data.
- 22 areas, one bottleneck view each, 2-3 fixes each.
- Next step: a pilot with one wellbeing services county.

TODO live URL (owner: Phương, N5) and repo URL

FirstLine: equal first steps, across Finland.

Backup

B1 to B6

B1

What our main number does and doesn't measure

- 6411: share of non-urgent GP visits kept more than 7 days after the need was assessed.
- All non-urgent visits, not only cancer. It counts visits, not people.
- Why still use it: early cancer symptoms are vague, so these patients land in the normal queue. **TODO** mentor answer to P1, only with recorded consent (Luan, N6)
- Indicator 13 shows when the hospital starts assessing a referral, not when the patient sees a specialist.
- 5033 is 0-3% everywhere, whole numbers: context only, not used for ranking.

THL Sotkanet 6411, 13, 5033; data/indicators.json limitations

B2

Grey means no public data, and we don't guess

- No public indicator for "GP recognises the symptoms": grey for every county.
- No referral data for Vantaa-Kerava or East, Central and West Uusimaa. Sotkanet region 599 is unnamed, so it is left out.
- Helsinki has referral data for 2025 only.
- Åland is excluded.
- Treatment access is out of scope. **TODO** figure for "data is flat" (indicator 5093) is not in data; Phương to confirm (N1)

Source: data/README.md

B3

Method and sources

- Sotkanet API to a static JSON snapshot; every row has a source_id.
- Status: each county's 2025 value vs Finland's 2025 value.
- The AI only writes text around fixed numbers; a validator drops any claim whose number or source is not in the data.
- Sotkanet 6411, 13, 5033 retrieved 13 Nov 2026.

/sources page screenshot

page is on main now; screenshot still to add

data/sources.json. **TODO** /sources screenshot (page merged in PR #10; see ../screenshots/09-sources.png) (Tuan, N3)

14 checked actions, matched to 3 bottlenecks

Bottleneck	Action	Result
Getting to a GP	Named GP (omalääkäri) they keep over time	Norway: 25% lower odds of dying in the year studied (observational)
Getting to a GP	Add GP capacity, named-GP model county-wide	Finland roadmap 2.5.1; effect not quantified
Getting to a GP	Nurses handle suitable first-contact visits	Cochrane, 18 trials: equal or better outcomes
Getting to a GP	Omaolo digital triage	Finland: 97.6% judged safe, 877 patients
GP recognition	Low referral threshold (about 3% cancer risk)	NICE NG12; effect not quantified
GP recognition	Safety-netting as standard	NICE NG12; effect not quantified
GP recognition	Direct CT access plus training	Denmark: no significant overall effect (36 vs 44 days)

data/evidence.json and docs/evidence/evidence.json (14 verified rows: 4 / 3 / 7); results shown honestly, including where waits did not fall. Owner and full source for each row are in the evidence table.

14 checked actions, matched to 3 bottlenecks

Bottleneck	Action	Result
GP to specialist	National referral criteria	Finland roadmap 2.5.2; effect not quantified
GP to specialist	Fast-track pathways with time limits	Denmark: median diagnostic interval 17 days shorter (adjusted)
GP to specialist	Funded pathways, regions report waits	Sweden: waits cut, regional gaps stayed
GP to specialist	Fast diagnostic route for vague symptoms	Denmark: 15-20% referred have cancer
GP to specialist	Rapid diagnostic centre pilot	England: 241 cancers in 2,961 referred (8%)
GP to specialist	Structured e-referral templates	Norway: referral quality score 18% higher
GP to specialist	E-consultation with a specialist	Ontario: answer in median 0.9 days

Sweden: ev-ref-sweden-svf. Denmark CT: ev-rec-direct-ct-training (previous page). Rows from docs/evidence/evidence.json.

B5

Exa layer: what is live, what is checked

- County news, the evidence finder under each action, and the judge question box.
- The Exa key stays on the server. Results are cached.
- If Exa is down: "Live search unavailable", and the checked table stays visible.
- **TODO** judge-box tests X/15 pass, median latency (Tuan, N10)

Live finds are labelled "found live - unverified" and kept apart from checked evidence.

B6

Likely questions

- **Is 6411 about cancer?** No, it is all non-urgent GP visits. It is the best public measure of the first step, and the app says so on every county page.
- **Did these actions really work?** We show the honest result: Sweden's pathways cut waits but regional gaps stayed; Denmark's direct CT access alone did not help significantly.
- **Who would pay or act?** The wellbeing services county. Every action names the owner, and two are already in the 2026-2031 cancer roadmap.

NICE and Käypä hoito are linked only, never copied. Source: docs/pitch/demo-script.md, docs/evidence/red-flags.md